

First Aid: CPR and the Unconscious Person

This guide covers how to respond when someone is unconscious, how to perform CPR (cardiopulmonary resuscitation), and how to place a person in the recovery position. Based on guidelines from the American Heart Association (2024), American Red Cross, NHS, and Mayo Clinic.

ACT IMMEDIATELY. When the heart stops, brain damage begins within 4 minutes. Every minute without CPR reduces the chance of survival by 7 to 10 percent. Do not wait.

SECTION 1: IS THE PERSON UNCONSCIOUS?

An unconscious person cannot be woken up by noise or touch. A sleeping person can.

To check:

Step 1. Tap both shoulders firmly and shout: "Are you okay? Can you hear me?"

Step 2. If no response, the person is unconscious.

Step 3. Shout for help immediately. Tell bystanders to call emergency services (112 in India and Europe, 911 in the US) and to bring an AED (defibrillator) if one is available.

Step 4. If you are alone, call emergency services yourself before starting CPR — unless the person is a child or infant, in which case give 2 minutes of CPR first, then call.

SECTION 2: CHECK FOR BREATHING

Tilt the person's head back gently by placing one hand on the forehead and lifting the chin with two fingers of your other hand. This opens the airway.

Look: Watch the chest for rise and fall.

Listen: Put your ear close to their mouth and nose and listen for breath sounds.

Feel: Feel for airflow against your cheek.

Do this for no more than 10 seconds.

GASPING IS NOT NORMAL BREATHING. Agonal breathing — occasional gasps or snoring sounds — is a sign of cardiac arrest, not normal breathing. If you see gasping, treat the person as if they are not breathing and begin CPR immediately.

SECTION 3: CPR — HANDS-ONLY (No Training Required)

If you are not trained in CPR, or not confident, perform Hands-Only CPR. This is highly effective and recommended by the American Heart Association for untrained bystanders.

Step 1. Place the person on their back on a firm, flat surface.

Step 2. Kneel beside the person at the level of their chest.

Step 3. Place the heel of one hand on the centre of the person's chest, directly between the nipples, on the lower half of the breastbone.

Step 4. Place your other hand on top and interlock your fingers. Keep your fingers raised off the chest — only the heel of the lower hand should be in contact.

Step 5. Position your shoulders directly above your hands and keep your arms straight (do not bend elbows).

Step 6. Push straight down at least 5 centimetres (2 inches) into the chest. Do not be afraid of pushing too hard — effective compressions must be deep.

Step 7. Release fully after each compression and let the chest rise back to its normal position. Do not lean on the chest between compressions.

Step 8. Compress at a rate of 100 to 120 per minute. This is fast — roughly two compressions per second. The rhythm of the song "Stayin' Alive" by the Bee Gees is exactly 100 beats per minute and can help you maintain the correct rate.

Step 9. Continue without stopping until: the person starts breathing normally, professional medical help takes over, an AED is ready to use, or you are physically unable to continue.

TAKE TURNS: Effective chest compressions are exhausting. If another person is available, switch every 2 minutes so compressions remain effective throughout.

SECTION 4: CPR WITH RESCUE BREATHS (Trained Rescuers)

If you are trained in CPR and confident in rescue breathing, combine compressions with rescue breaths. The ratio is 30 compressions followed by 2 rescue breaths.

After 30 compressions:

Step 1. Tilt the person's head back and lift the chin to open the airway.

Step 2. Pinch the nose firmly shut with your thumb and forefinger.

Step 3. Take a normal breath (not a deep breath).

Step 4. Make a complete seal over the person's mouth with your mouth.

Step 5. Breathe out steadily for about 1 second. Watch for the chest to rise. If the chest rises, the breath went in successfully.

Step 6. Remove your mouth, allow the chest to fall, then give the second breath.

Step 7. If the chest does not rise on the first breath, re-tilt the head and try again. If it still does not rise, the person may have something blocking their airway.

Step 8. Return immediately to 30 compressions. Repeat the cycle of 30:2.

IF UNCOMFORTABLE WITH RESCUE BREATHS: Hands-only CPR (compressions only) is significantly better than no CPR at all. Do not delay starting compressions because you are uncertain about rescue breaths.

SECTION 5: CPR FOR CHILDREN (1 year to puberty)

The process is the same as adult CPR with these differences:

- Use only ONE hand for compressions, or two fingers for very small children.
- Push down approximately 5 centimetres (2 inches) or one-third of the depth of the chest.
- For children, begin with 5 rescue breaths before starting compressions.
- Maintain the same 30:2 ratio of compressions to breaths.
- Rate remains 100 to 120 compressions per minute.

SECTION 6: CPR FOR INFANTS (Under 1 year)

- Use two fingers (index and middle) in the centre of the chest just below the nipple line.
- Push down approximately 4 centimetres (1.5 inches).
- Begin with 5 gentle rescue breaths covering both the mouth AND nose with your mouth.
- Then 30 compressions followed by 2 breaths.
- Rate: 100 to 120 compressions per minute.

SECTION 7: THE RECOVERY POSITION (Unconscious but Breathing)

If the person IS breathing but unconscious, do NOT perform CPR. Instead, place them in the recovery position to prevent choking if they vomit.

Step 1. Kneel beside the person.

Step 2. Place the arm nearest to you at a right angle to their body, elbow bent with the palm facing upward.

Step 3. Bring their far arm across their chest and hold the back of their hand against their near cheek.

Step 4. With your other hand, pull up their far knee so the foot is flat on the ground.

Step 5. Keeping the person's hand against their cheek, pull the bent knee toward you to roll them onto their side.

Step 6. Tilt the head back gently to make sure the airway stays open.

Step 7. Monitor breathing continuously. If breathing stops at any point, roll the person onto their back and start CPR immediately.

SECTION 8: DROWNING — IMPORTANT DIFFERENCES

For a person rescued from drowning:

Rescue breaths first: Give 5 rescue breaths before starting chest compressions. The primary problem is lack of oxygen, not cardiac arrest. This is different from standard cardiac arrest CPR.

Do not do in-water CPR unless you are a trained lifeguard — it is ineffective in moving water.

Get the person out of the water first, then begin CPR on a firm, flat surface.

Continue CPR — victims of cold-water drowning have survived after extended periods without a pulse. Do not give up until emergency professionals take over.